

STILNOX®

COMPOSITION

Each scored film-coated tablet contains:

Zolpidem tartrate..... 10 mg

PHARMACEUTICAL FORM

Scored film-coated tablets.

CLINICAL PARTICULARS

Therapeutic indications

Indications are limited to treatment of severe sleep disorders in adults with:

- Transient insomnia
- Short-term insomnia

Posology and method of administration

Posology

Dose

The treatment should be taken as a single dose and not be re-administered during the same night.

The recommended daily dose for adults is 10 mg to be taken immediately at bedtime.

Treatment should be started at the lowest effective dose and the maximum dose must not exceed 10 mg.

Elderly or debilitated subjects

As elderly or debilitated patients are especially sensitive to the effects of zolpidem, the recommended dose in this population is 5 mg (i.e. 1/2 a tablet).

Liver failure

As the clearance and metabolism of zolpidem are reduced in liver failure, treatment should begin at 5 mg per day in these patients, with particular caution being exercised in elderly patients.

Under no circumstances should the daily dose exceed 10 mg.

Stilnox may be prescribed for use continuously or as required, depending on the signs and symptoms.

Pediatric population

The safety and efficacy of zolpidem in children and adolescents under 18 years of age have not been demonstrated. Therefore, use of zolpidem is not recommended in this population. The available evidence from placebo-controlled clinical trials is presented in section Pharmacodynamic properties.

Duration

As with all hypnotics, long-term use of zolpidem is not recommended. Treatment should be as short as possible, and should not exceed four weeks, including the tapering-off period (*see section Special warnings and special precautions for use*).

The duration of treatment must be explained to the patient:

- 2 to 5 days for transient insomnia (when traveling, for example),
- 2 to 3 weeks for short-term insomnia (following a serious event, for example).

Very short-term treatments do not require a tapering-off period.

Extension beyond the maximum treatment period should not take place without re-evaluation of the patient's status, since the risk of abuse and dependence increases with the duration of treatment.

Method of administration

Oral use.

Contraindications

This medicinal product must never be used in patients with:

- hypersensitivity to the active substance or to any of the ingredients in the product,
- severe respiratory failure,
- sleep apnea syndrome,
- severe, acute or chronic liver failure (due to the risk of encephalopathy),
- history of complex sleep patterns after the use of zolpidem (see section Special warnings and special precautions for use),
- myasthenia.

Special warnings and special precautions for use

Anaphylaxis (severe allergic reaction) and angioedema (severe facial swelling) which can occur as early as the first time the product is taken

Complex sleep – related behaviors which may include sleep driving, making phone calls, preparing and eating food while asleep

Special warnings

PHARMACOLOGICAL TOLERANCE

The sedative or hypnotic effect of benzodiazepines and related drugs may gradually decrease, despite using the same dose, if administered over several weeks.

ABUSE AND DEPENDENCE

Any treatment with zolpidem, particularly for long periods, may lead to the development of abuse and/or physical and psychological drug dependence.

The risk of dependence increases with the dose and duration of treatment.

Cases of dependence have been reported more frequently in patients treated with zolpidem for more than 4 weeks. The risk of abuse and dependence is also higher in patients with a history of psychiatric disorders and/or alcohol and illicit substance dependence, or drug abuse. Zolpidem should be used with extreme caution in patients with current or a history of alcohol or drug abuse.

Drug dependence may occur at therapeutic doses and/or in patients with no specific risk factors.

WITHDRAWAL SYNDROME

When treatment is discontinued, withdrawal syndrome may occur.

Some symptoms are frequent and may appear to be unremarkable: insomnia, headache, extreme anxiety, myalgia, muscle tension and irritability.

Other symptoms are rarer: agitation or even confusion, paresthesia of the extremities, hypersensitivity to light, noise or physical contact, depersonalization, derealization, hallucinatory phenomena, delirium, convulsions.

Withdrawal symptoms may occur within days of treatment discontinuation. For short-acting benzodiazepines, and particularly when given at high doses, symptoms may even appear between two doses.

Concomitant use of several benzodiazepines, whether for treatment of anxiety or sleep disorders, may increase the risk of drug dependence.

REBOUND EFFECT

This transient syndrome may occur as an exacerbation of the insomnia which was the initial reason for treatment with benzodiazepines and related drugs.

PSYCHOMOTOR FUNCTION DISORDERS

Like other sedatives/hypnotics, zolpidem has Central Nervous System (CNS) depressant effects.

Psychomotor function disorders may occur within hours of administration.

The risk of psychomotor function disorders, including impaired driving ability, is increased if:

- this medicine is taken less than 8 hours before performing activities that require mental alertness (*see section Effects on ability to drive and use machines*);
- a dose higher than the recommended dose is taken;
- the medicine is co-administered with other CNS depressants or with other drugs that increase zolpidem blood levels, or with alcohol or illicit drugs (*see section Interaction with other medicinal products and other forms of interaction*).

Zolpidem should be taken as a single dose immediately at bedtime and not be re-administered during the same night.

AMNESIA

Anterograde amnesia may occur within hours of administration.

In order to reduce this risk, patients should ensure that they will be able to sleep uninterrupted for 8 hours (*see section Undesirable effects*)

BEHAVIORAL DISTURBANCES

In some subjects, benzodiazepines and related drugs may cause a syndrome combining varying degrees of impaired consciousness with behavioral and memory disorders.

The following may be observed:

- exacerbated insomnia, nightmares, agitation, nervousness,
- delirium or delusions, hallucinations, confuse-oneiric state, psychotic-like symptoms,
- loss of inhibition with impulsiveness,
- euphoria, irritability,
- anterograde amnesia,
- suggestibility.

This syndrome may be accompanied by disorders potentially dangerous to the patient or to others, such as:

- abnormal behavior for the patient,
- self-aggression or aggression towards others, particularly if the family and friends try to prevent the patient from doing what he/she wants,
- automatic behavior with post-event amnesia.

These symptoms require treatment discontinuation.

SOMNAMBULISM AND ASSOCIATED BEHAVIOR:

Complex sleep behaviours, including sleepwalking and other associated behaviours such as sleep driving, preparing and consuming food, making phone calls or having sex, accompanied by post-event amnesia, have been reported in patients who had taken zolpidem and were not fully awake. These events may occur after the first or any subsequent use of zolpidem. Treatment with zolpidem should be stopped immediately if a patient exhibits complex sleep behaviour, because of the risk to him-/herself and those around him/her (see section Contraindications). Consumption of alcohol and other central nervous system depressants with zolpidem appears to increase the risk of such behaviours, as does the use of zolpidem at doses exceeding the maximum recommended dose.

Discontinuation of zolpidem should be strongly considered for patients who report such behaviors, due to the risk to the patient and others. (*see sections Interaction with other medicinal products and other forms of interaction, and Undesirable effects*).

RISK OF ACCUMULATION

Benzodiazepines and related drugs (like all medicinal products) remain in the body for about 5 elimination half-lives (*see section Pharmacokinetic properties*).

In the elderly or in patients with renal or hepatic insufficiency, the elimination half-life may be considerably longer. Following administration of repeated doses, steady-state concentrations for the drug or its metabolites are achieved much later and are much higher. The efficacy and the safety of the drug can only be assessed once steady-state concentrations have been achieved.

Dose adjustment may be necessary (*see section Posology and method of administration*). This is not expected with zolpidem in patients with kidney failure given the metabolic profile of the drug (*see section Pharmacokinetic properties*).

RISK OF CONCOMITANT USE WITH OPIOIDS

Concomitant use of zolpidem with other opioids can cause sedation, respiratory depression, coma and death. Given these risks, concomitant prescription of opioids and sedatives, e.g. benzodiazepines and other related drugs such as zolpidem, should be limited to patients for whom there are no alternative treatment options.

If zolpidem and opioids have been prescribed concomitantly, the lowest effective dose should be prescribed and duration of treatment should be as short as possible. Patients must be closely monitored for any signs and symptoms of respiratory depression and sedation. Therefore, it is highly recommended that patients and those around them be made aware of these symptoms

ELDERLY SUBJECTS

Benzodiazepines and related drugs should be used with caution in elderly subjects since there is a risk of sedation and/or muscle-relaxant effects which may promote falls, which often have serious consequences in this population, and because the incidence of behavioral disturbance is higher in these patients.

LIVER FAILURE

Dose adjustment is required due to the risk of accumulation (*see section Posology and method of administration*).

Zolpidem should not be used in patients with severe liver failure due to the risk of encephalopathy (*see section Posology and method of administration, Contraindications, and Undesirable effects*).

RELATED TO THE PRESENCE OF AN EXCIPIENT WITH KNOWN EFFECT

This medicinal product contains lactose. It is not recommended for use in patients with galactose intolerance. Lapp lactose deficiency or glucose-galactose malabsorption syndrome (rare hereditary diseases).

This medicinal product contains less than 1 mmol (23 mg) of sodium per tablet, i.e. it is essentially <<sodium-free>>.

Special precautions for use

Extreme caution is recommended in patients with a history of alcoholism or other addictions, whether to medicinal products or other substances (*see section Interaction with other medicinal products and other forms of interaction*).

In all cases, insomnia should be assessed routinely, and the underlying causes treated, before prescribing a hypnotic agent.

Insomnia may be a sign of an underlying physical or psychiatric disorder. The clinical diagnosis should be reassessed if insomnia persists or worsens after a short period of treatment.

DURATION OF TREATMENT

Patients must be clearly informed of duration of treatment depending on the type of insomnia (*see section Posology and method of administration*).

SUICIDALITY - DEPRESSION - MAJOR DEPRESSIVE EPISODE

Some epidemiological studies show an increased incidence of suicidal ideation, suicide attempts and suicides in patients with or without depression, treated with benzodiazepines and other hypnotics, including zolpidem. However, the causal link has not been established.

Insomnia may be a symptom of depression; depression must therefore be treated. If insomnia persists, the patient should be reassessed.

In patients with a major depressive episode:

Benzodiazepines and related drugs should not be prescribed alone, since this allows the depression to progress independently, along with persistent or enhanced risk of suicide.

Suicidal tendencies may be present, therefore the lowest possible amount of zolpidem should be supplied to these patients (prescription and supply) to avoid the possibility of intentional overdose.

METHODS FOR PROGRESSIVE DISCONTINUATION OF TREATMENT

These should be clearly explained to the patient.

In addition to the need to taper the doses, patients should be warned of the risk of rebound effect, so as to minimize any insomnia which may arise because of the symptoms resulting from discontinuation, even if it is gradual.

Patients must be informed of possible discomfort during the tapering-off period.

PAEDIATRIC POPULATION

Use of Stilnox 10 mg scored film-coated tablets in children is not recommended since no clinical studies have been performed in this population.

ELDERLY SUBJECTS

The risk of accumulation requires dose adjustment (*see section Posology and method of administration*).

PATIENTS WITH RESPIRATORY FAILURE

The depressant effect of benzodiazepines and related drugs should be taken into account when prescribed to patients with respiratory failure (particularly since anxiety and restlessness may be warning signs of respiratory decompensation, requiring transfer to an intensive care unit).

PATIENTS WITH LONG QT SYNDROME

Under experimental conditions using very high concentrations and pluripotent stem cells, an in vitro cardiac electrophysiological study showed that zolpidem may reduce the permeability of the hERG potassium channels. The potential consequence of this effect in patients with congenital long QT syndrome is not known. As a precautionary measure, the benefit/risk ratio of treatment with zolpidem in patients with congenital long QT syndrome should be carefully assessed.

Interactions with other medicinal products and other forms of interaction

Hypnotic agents

Currently prescribed hypnotic agents are either benzodiazepines and related products (zolpidem, zopiclone) or H1 antihistamines. In addition to increased sedation when prescribed with other CNS depressants, or in the event of alcohol intake, the possible increase in the respiratory depressant effect when co-administered with morphine-like substances, other benzodiazepines, or phenobarbital should also be taken into account for benzodiazepines, especially in the elderly.

Central nervous system depressants

The fact that many drugs or substances can have additive depressant effects on the central nervous system and contribute to a decrease in alertness must be taken into account. These drugs include morphine derivatives (analgesic agents, antitussive agents and replacement therapies), neuroleptic agents, barbiturates, benzodiazepines, anxiolytic agents other than benzodiazepines (such as meprobamate), hypnotic agents, sedative antidepressants (amitriptyline, doxepin, mianserin, mirtazapine, trimipramine), sedative H1-antihistamines, centrally-acting antihypertensive agents, baclofen, thalidomide, antiepileptic drugs and anesthetics

Inadvisable combinations

+ Alcohol (beverage or excipient)

Alcohol enhances the sedative effect of benzodiazepines and related drugs. Impaired alertness may make driving or using machines dangerous.

Patients should not consume alcoholic beverages or medicinal products containing alcohol.

+ Fluvoxamine

Co-administration of fluvoxamine may increase blood levels of zolpidem. Concomitant use is not recommended.

+ CYP450 inhibitors and inducers

Co-administration of ciprofloxacin may increase blood levels of zolpidem. Concomitant use is not recommended

+ St. John's Wort

St. John's Wort has been shown to have a pharmacokinetic interaction with zolpidem. Mean zolpidem C_{max} and AUC levels are lower (by 33.7% and 30.0%, respectively) when co-administered with St. John's Wort, compared to the levels with single dose administration. Co-administration of St. John's Wort may decrease blood levels of zolpidem. Concurrent use is not recommended.

Combination requiring precautions for use

+ Rifampicin

Decreased plasma concentrations and reduced efficacy of zolpidem due to increased hepatic metabolism. Clinical monitoring is required. If necessary, another hypnotic agent may be used.

Combinations to be taken into consideration

+ Other central nervous system depressants

Possible increase in next-day drowsiness and psychomotor impairment. Impaired alertness may make driving vehicles and using machines dangerous (*see sections*

Special warnings and precautions for use and Effects on ability to drive and use machines).

Isolated cases of visual hallucinations have been reported in patients taking zolpidem with antidepressants including bupropion, fluoxetine, sertraline and venlafaxine.

+ Barbiturates, morphine derivatives:

Increased risk of respiratory depression, which may be fatal in the event of an overdose.

+ Buprenorphine

When buprenorphine is used as a replacement therapy: increased risk of potentially fatal respiratory depression. The benefit/risk ratio of this combination should be carefully assessed. Patients must be informed of the need to comply with prescribed doses.

+ Clozapine

Increased risk of collapse with respiratory and/or cardiac arrest.

+ Opioids

Concomitant use of opioids and sedatives, e.g. benzodiazepines and related drugs such as zolpidem, increases the risk of sedation, respiratory depression, coma and death due to the increased central nervous system depressant effect. Doses and duration of concomitant treatment must be limited

+ Clarithromycin, erythromycin, telithromycin

Mild increased sedative effects of zolpidem.

+ Ketoconazole, itraconazole, voriconazole

Mild increased sedative effects of zolpidem.

+ Nelfinavir, ritonavir-boosted protease inhibitors

Mild increased sedative effects of zolpidem.

When zolpidem was administered with ranitidine, no significant pharmacokinetic interactions were observed.

Pregnancy and lactation

Pregnancy

There is no evidence from extensive data collected during cohort studies that exposure to benzodiazepines during the first trimester of pregnancy results in any malformative effects. However, in some epidemiological case-control studies, an increased incidence of cleft lip and palate was observed with benzodiazepines. According to these data, the incidence of cleft lip and palate would appear to be less than 2 per 1000 in neonates exposed to benzodiazepines *in utero*, compared to an expected ratio of 1 per 1000 in the general population.

Decreased fetal movement (DMF) and fetal heart rate changes have been described with administration of high benzodiazepine doses during the second and/or third trimester of pregnancy. Benzodiazepine treatment at the end of pregnancy, even at low doses, may cause signs of exposure in the neonate, such as axial hypotonia and difficulty suckling, leading to poor weight gain. Although these signs are reversible, they may last for 1 to 3 weeks depending on the half-life of the benzodiazepine prescribed. At high doses,

respiratory depression or apnea and hypothermia may occur in the neonate. Furthermore, neonatal withdrawal syndrome may occur, even if there are no signs of exposure. This is characterized in particular by overexcitability, agitation and tremor in the neonate, occurring some time after delivery. The time to onset depends on the elimination half-life of the medicinal product, and may be significant if the elimination half-life is long.

Based on these data, as a precautionary measure, use of zolpidem is not recommended during pregnancy, regardless of the trimester.

Women of child-bearing potential under zolpidem treatment should be instructed to contact their physician if they plan a pregnancy or if they are in the early stages of pregnancy, in order to reassess the need for treatment.

At term, if zolpidem treatment is absolutely necessary, high doses should be avoided and the effects described above taken into account during neonatal monitoring.

Lactation

Use of zolpidem is not recommended in breast-feeding women

Effects on ability to drive and use machines

Stilnox 10 mg scored film-coated tablets have major influence on the ability to drive and use machines.

Patients who drive and use machines should be warned that, as with other hypnotics, there may be a possible risk of drowsiness, longer reaction time, dizziness, torpor, blurred/double vision and reduced alertness as well as impaired driving ability the morning after administration.

In order to minimize this risk, at least 8 hours of uninterrupted sleep is recommended between taking zolpidem and driving, using machinery and working at heights.

Driving ability impairment and behaviors such as falling asleep while driving have occurred in patients taking zolpidem alone at therapeutic doses.

Furthermore, co-administration of zolpidem with alcohol and other CNS depressants increases the risk of such behaviors. Patients should be warned not to drink alcohol or take other psychoactive substances during treatment with zolpidem.

Undesirable effects

Adverse effects are classified according to incidence using the following classification:

Very common ($\geq 1/10$); common ($\geq 1/100$ to $< 1/10$); uncommon ($\geq 1/1000$ to $< 1/100$); rare ($\geq 1/10\ 000$ to $< 1/1000$); very rare ($< 1/10\ 000$) not known (cannot be estimated from the available data)

These effects are related to the dose administered and the individual sensitivity of each patient.

Immune system disorders

- Not known: angioneurotic oedema

Psychiatric disorders (see section Special warnings and special precautions for use)

- Common: hallucinations, agitation, nightmares, depression (see section Special warnings and special precautions for use).
- Uncommon: confusional state, irritability, restlessness, aggression, somnambulism (see section Special warnings and special precautions for use), euphoric mood, complex sleep behaviours (see section Special warnings and special precautions for use).
- Rare: libido disorder.
- Very rare: physical and psychological dependence, even at therapeutic doses, with withdrawal syndrome or rebound effect that may occur on treatment discontinuation.
- Not known: behavioral disturbances, anger, delirium (see section Special warnings and special precautions for use).

Nervous system disorders (see section Special warnings and special precautions for use)

- Common: impaired alertness or even drowsiness (particularly in the elderly), headache, vertigo, insomnia, cognitive disorders such as anterograde amnesia, which may occur at therapeutic doses, the risk increasing in proportion to dose.
- Uncommon: paresthesia, tremor, attention and speech disorders.
- Rare: impaired consciousness.
- Not known: ataxia, tension.

Eye disorders

- Uncommon: diplopia, blurred vision.
- Rare: vision impairment.

Respiratory, thoracic and mediastinal disorders

- Very rare: respiratory depression (see section Special warnings and special precautions for use).

Gastrointestinal disorders

- Common: diarrhea, nausea, vomiting, abdominal pain.

Hepatobiliary disorders

- Uncommon: elevated liver enzymes
- Rare: hepatocellular lesions, cholestatic or mixed liver injury (see section Posology and method of administration, Contraindications, and Special warnings and special precautions for use).

Metabolism and nutrition disorders

- Uncommon: appetite disorders.

Skin and subcutaneous tissue disorders

- Uncommon: skin rash, pruritus.
- Rare: urticaria.

Musculoskeletal and connective tissue disorders

- Uncommon: arthralgia, myalgia, muscle spasm, muscle hypotonia.

Infections and infestations

- Common: upper and lower respiratory tract infections.

General disorders and administration site conditions

- Common: asthenia.
- Rare: balance disorders or even falls.

Reporting suspected adverse effects

Reporting suspected adverse effects after the drug has been approved is important. This makes it possible to continuously monitor the benefit/risk ratio of the drug. Healthcare professionals are asked to report all suspected adverse effects via the national reporting system.

Overdose

Overdose can be life-threatening, particularly multiple overdose involving other central nervous system depressants (including alcohol). In the case of zolpidem, the prognosis has always been positive for single-agent intoxication with doses of zolpidem of up to 400 mg.

If massive amounts of zolpidem are taken alone or concomitantly with other central nervous system depressants (including alcohol), the main sign of overdose is central nervous system depression, which may range from drowsiness to coma, depending on the amount ingested.

In non-serious cases of overdose, symptoms include mental confusion and lethargy.

In more serious cases, signs include ataxia, hypotonia, hypotension, respiratory depression and, in very rare cases, death.

If oral overdose occurred less than one hour before, vomiting should be induced in conscious patients; otherwise gastric lavage must be performed with protection of the airways. After this time point, administration of activated charcoal may be useful in reducing absorption.

Specific monitoring of cardiac and respiratory functions in a specialized setting is recommended.

Administration of flumazenil may be useful for diagnosis and/or treatment of accidental or intentional overdose with benzodiazepines.

Antagonism of the effects of benzodiazepines by flumazenil may promote neurological disorders (seizures), particularly in epileptic patients.

Zolpidem is not dialyzable.

PHARMACOLOGICAL PROPERTIES

Pharmacodynamic properties

HYPNOTICS AND SEDATIVES, ATC code: N05CF02

(N: Central Nervous System)

Zolpidem is a hypnotic imidazopyridine related to benzodiazepines with a pharmacodynamic activity qualitatively similar to that of other compounds in the same class.

- muscle relaxant,
- anxiolytic,
- sedating,
- hypnotic,
- anticonvulsant,
- amnesic.

Experimental studies have evidenced sedating effects at doses lower than those required to obtain anticonvulsant, muscle relaxant and anxiolytic effects.

These effects are related to specific agonist action at a central receptor belonging to the GABA-omega (BZ1 and BZ2) “macromolecular receptor complex”, modulating the opening of the chloride ion channel.

Zolpidem binds selectively onto the omega 1 subtype (or BZ1).

In man, zolpidem shortens time to falling asleep, reduces the number of night awakenings, increases total sleep duration and improves quality of sleep. These effects are associated with a characteristic electroencephalographic profile, different to that induced by benzodiazepines. Night-sleep recording studies have demonstrated that zolpidem prolongs stage II sleep and deep sleep (III and IV). At recommended doses, zolpidem does not affect total duration of paradoxical sleep (R.E.M.).

In randomized trials, zolpidem was shown to be effective only at the 10 mg dose.

In a randomized double-blind trial in 462 healthy volunteers under 65 years of age with transient insomnia, zolpidem decreased the mean time to fall asleep compared to placebo by 10 minutes in the 10 mg group and by 3 minutes in the 5 mg group.

In a randomized double-blind trial in 114 patients under 65 years of age with chronic insomnia, zolpidem decreased the mean time to fall asleep compared to placebo by 30 minutes in the 10 mg group and by 15 minutes in the 5 mg group.

In some patients, a dose of less than 5 mg may prove to be effective.

Children and adolescents

A randomized placebo-controlled study in 201 children and adolescents aged 6-17 years with insomnia associated with Attention Deficit Hyperactivity Disorder (ADHD) failed to demonstrate efficacy of zolpidem 0.25 mg/kg/day (with a maximum of 10 mg/day) as compared to placebo. Psychiatric and neurological disorders were the adverse events most frequently observed in patients treated with zolpidem compared to the placebo group, and included dizziness (23.5% versus 1.5%), headache (12.5% versus 9.2%), and hallucinations (7.4% versus 0%).

Consequently, and in the absence of any further studies, zolpidem must not be prescribed in this population

Pharmacokinetic properties

Absorption

After oral administration, the bioavailability of zolpidem is about 70%, reaching peak plasma concentration between 0.5 to 3 hours.

Distribution

At therapeutic doses, the pharmacokinetics are linear. The degree of plasma protein binding is about 92%. The distribution volume in adults is $0.54 + 0.02$ l/kg.

Metabolism and elimination

Zolpidem is excreted in the form of inactive metabolites (hepatic metabolism), mainly in the urine (about 60%) and faeces (about 40%).

It has no inducing effect on liver enzymes.

Plasma elimination half-life is about 2.4 hours (0.7 – 3.5 hours).

At-risk populations

- Liver clearance is reduced in elderly patients. Peak plasma concentrations are approximately 50% higher in this population and the half-life is not significantly prolonged (mean of 3 hours). The volume of distribution is reduced to 0.34 ± 0.05 l/kg.
- Renal clearance is moderately decreased in patients with kidney failure, regardless of whether they are dialyzed or not. The other kinetic parameters remain unchanged. Zolpidem is not dialyzable.
- In patients with liver failure, bioavailability of zolpidem is increased. Clearance is substantially reduced and elimination half-life prolonged (approximately 10 hours).

PHARMACEUTICAL PARTICULARS**Storage**

Store at or below 30°C,

Shelf-life

3 years

Pack size

Box of 14 or 20 tablets in blister packs.

Manufacturer

Sanofi Winthrop Industrie
30-36, avenue Gustave Eiffel
37100 Tours, France

DATE OF REVISION:

Dec 2025

(CCDS V17 + Long QT syndrome updated based on France SmPC Feb 2024)